



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : هبه عادل محمود على

العنوان : مساكن ايبسكو المقطم
القاهرة

الحي / المدينة : المقطم

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 22-09-2026

رقم البطاقة : 28701042301365 رقم التليفون : 01060781513

العمر (تاريخ الميلاد) : 04-01-1987 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة :
الوظيفة :
الوظيفة :

التوقيع :
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التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: هبه عادل محمود علي National ID: 28701042301365	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Mammography and breast ultrasound
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٠/٠٦ ٢٨٧٠١٠٤٢٣٠١٣٦٥

حاصله على دبلوم تجارة



أنثى مسلمة متزوجة

شعبان عيد حسين محمود

البطاقة سارية حتى ٢٠٢٧/٠٦/٢٩



جمهورية فلسطين العربية
بطاقة تحقيق الشخصية

هبة
عادل محمود على
مساكن اسبيكو ب-١٣ م ٢
المقطم - القاهرة

٢٨٧٠١٠٤٢٣٠١٣٦٥

IK9376032

Patient Name: Heba Adel Mahmoud Ali	Date : 28/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 38 Year(s)
Medical ID: 3444/2024	Pat. ID: 79298

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Status post left mastectomy showing :

(I) Chest:

- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

Compared to the last study dated 13-2-2025 ,the current study revealed:

- The liver is average in size showing homogenous CT density with **complete resolution** of the right hepatic segment VIII focal lesion.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Still noted bulky uterus with complete resolution of the left ovarian lesion ,for sonographic assessment.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.

- The skeletal survey shows:
 - o No lytic osseous lesions.
 - o Stable right femoral head and left iliac sclerotic bone lesions

OPINION:

- **Follow up status showing the above detailed changes.**
- **No CT evidence of pulmonary deposits.**

Dr.Sara Mahmoud, MD

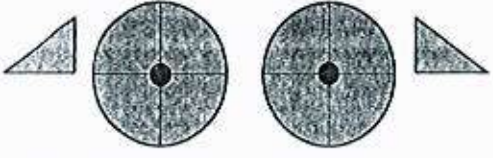
Dr.Sara Ali

Dr.Sara Ali

MDT FORM 19/8/2025 G

Name	د. خالد محمود	MRN	3444/2024
Age	38	ID	
Menopausal status		Phone	01060781513
NRS SCORE		Marital status	Married
BMI		Family history	-
CO morbidity	NAD	REASON OF MDT	

Clinical Data

BX: IDC	R	L
ER: 018	PR: 016	HER 2: 3
Ki67:		
Sonomammography:		
Mets Workup:	TNM	

Recommendations: Post NACT L+MRM
CPR (01/17) LN
No residual invasive tumor
5% Intermediate DCIS

MDT OUTCOME & PLAN: Adjuvant Herceptin to complete one year till cycle 17.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. Mohamed El-Sayed					

PATIENT NAME: HEBA ADEL MAHMOUD ALI

GENDER: Female

DATE : 25-May-2025 12:49 PM

PATIENT ID: 10124251107

AGE: 38 y

ORDER: 3394780

WHOLE BODY FDG PET-CT EXAMINATION

Clinical background:

- 38-year-old female patient.
- Presented with left breast cancer, received chemotherapy (last cycle ended 10/04/2025), currently on Herceptin.
- Weight = 114 Kg.
- BL glucose level = 102 mg/dl.

Reason of study:

- Metastatic workup.
- Radiopharmaceutical:
- F-18-FDG 8.5 mCi injected.

Technique:

- Approximately one hour after IV injection of the radiopharmaceutical, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- Reference hepatic activity (SUV max)~ 2.60

Findings:

Reviewing PET, CT and fused PET-CT images:-

I- Breasts and axillary regions:

- Left breast low grade metabolically active mild skin thickening with no definite metabolically active mass lesion.
- No appreciable FDG uptake in few left axillary level I and II lymph nodes, the largest measuring about 1 x 1.7 cm.
- No metabolically active (FDG avid) contralateral breast masses or

PATIENT: HEBA ADEL MAHMOUD ALI, 10124251107, Female, 38 y

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- Low grade metabolically active left breast skin thickening with no definite mass lesion with no appreciable metabolic activity in ipsilateral axillary lymphadenopathy.
- Non FDG avid few bilateral pulmonary nodules for follow up.
- No other metabolically active sizable neoplastic lesions in the rest of the surveyed body.

Reported By,

Dr. Fifi Samy, MSc

Dr. Mohamed Mohsen, MD

Dr. ElShaymaa Hussein, Nuclear Medicine Consultant

26-May-2025 04:29 PM

PATIENT: HEBA ADEL MAHMOUD ALI, 10124251107, Female, 38 y

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PATIENT NAME: HEBA ADEL MAHMOUD ALI

GENDER: Female

DATE : 22-Apr-2025 02:46 PM

PATIENT ID: 1012425

AGE: 38 y

ORDER:

3336160,3336161,3338:

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER**TECHNIQUE:**

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 90 ml.

FINDINGS:**1-CHEST**

- Left UOQ parenchymal distortion.
- Multiple prominent left level I,II axillary lymph nodes, the largest reaching cortical thickness of 5mm.
- Bilateral few tiny pulmonary nodules, average size 3-4mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.
- Anterior mediastinal soft tissue density likely thymic remnants.
- Small sliding hiatus hernia

2- ABDOMEN AND PELVIS: compared to last CT done 13.02.2025

- The liver is of average size with homogenous CT texture showing rather stationary hepatic dome hypodense focal lesion, being less appreciated in delayed phase measuring about 10 x 6 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation..
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.

PATIENT: HEBA ADEL MAHMOUD ALI, 10124251107, Female, 38 y

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- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Normal appearance of the aerodigestive air column with no parapharyngeal masses.
- Normal appearance of the vocal cords, both pyriform sinuses and ary-epiglottic folds. No glottic, supra- or infraglottic masses.
- Intact laryngeal skeleton.
- No FDG avid thyroid nodules.
- Mild right maxillary and bilateral ethmoidal sinusitis.

III. CHEST:

- Non FDG avid few bilateral pulmonary nodules, the largest at left upper lung lobe measuring about 4 mm.
- Low grade metabolically active anterior mediastinal soft tissue sheet likely thymic hyperplasia.
- No metabolically active mediastinal lymph nodes.
- No metabolically active pleural effusion or masses.
- Average cardiac size. No pericardial effusion detected.

IV. ABDOMEN AND PELVIS:

- The liver is average in size with homogenous CT texture. No definite CT evidence of metabolically active (FDG avid) focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal appearance of the pancreas, spleen, both kidneys and both suprarenals (on non contrast CT basis) with No FDG avid focal lesions.
- No metabolically active abdominal or pelvic lymphadenopathy.
- Under filling of the urinary bladder with no filling defects or diverticular outpouching.
- No ascites.
- Focal increased endometrial FDG uptake with SUV max ~ 5.78 near the fundus: for TVUS correlation.

V. Scanned axial and proximal appendicular skeleton:

- No metabolically active (FDG avid) osseous lesions.
- Non FDG avid right femoral head sclerotic focus likely bone island.
- Non FDG avid left iliac wing sclerotic lesion and multiple vertebral lucent foci.

PATIENT: HEBA ADEL MAHMOUD ALI, 10124251107, Female, 38 y

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Patient Name : الأستاذة / هبة عادل محمود على	Request No. : 13002736
Patient No. : 28701042301365	Request Date : 23/09/2024
Path No. : 3559/24	Verification Date : 07/10/2024
Sex : Female	Financial Category : مريض - عيادة
Age : 37 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 22/9/2024.

Gross Examination:

- 4 grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: No.
- * Angiolymphatic invasion: No.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma G II with focal DCIS.

For further IHC (E- cadherin, ER, PR, Her2, Ki67).

Signature

Prof. Dr. Lubna Omar.

Dr. Menna Nabil.

Verified by :

DR.Menna Nabil





Patient Name: Heba Adel Mahmoud Ali	Date : 05/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 38 Year(s)
Medical ID: 3444/2024	Pat. ID: 79298

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast malignancy on NACTH

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows near total resolution of the forenoted UOQ few spiculated irregular mass lesions, of high density.
- Complementary ultrasound revealed residual subtle areas of parenchymal distortion at UOQ with no measurable masses.
- Regressive course of the pathological left axillary lymph node.
- Free supra and infraclavicular regions.
- **Right breast:**
- Normal skin thickness, and nipple-areola complex of the right breast.
- No right breast solid or cystic mass lesions
- Nonspecific right axillary LN.

Opinion:

- Regressive course of the left breast UOQ malignant looking mass lesions and ipsilateral axillary lymph nodes denoting complete response (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Dr. Mohamed Mohsen, M.D.